NORTHGATE MULTISPECIALITY HOSPITAL

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OUTPATIENT COMPREHENSIVE FOLLOW-UP & METABOLIC EVALUATION

Patient Name: Mr. ARJUN MEHTA	**UHID / MRN:** NG-0098731	**Evaluation Date:** 19/01/2027
Age / Sex: 58 Years / Male	**IPD Ref:** IP/2026/045512 (Discharged 19/10/2026)	**Follow-up Interval:** 3 Months Post-Discharge
Consultant: Dr. Neha Example, MD, DM (Endocrinology)	**Clinic:** Endocrinology & Diabetes Care OPD	**Vitals: BP 120/78 mmHg, BMI 26.2 kg/m²**

CLINICAL INTERVAL HISTORY & ASSESSMENT

58-year-old male returns for scheduled 3-month review following October 2026 hospitalisation for severe hyperglycaemia with ketosis. Patient reports full adherence to prescribed metformin (500mg BID), titrated insulin regimen, and lifestyle modifications (daily 35-min brisk walking, DASH dietary protocol). Reports zero hypoglycaemic episodes. Fatigue, polyuria, and polydipsia have completely resolved. Weight stable at 75 kg (BMI reduced from 27.8 to 26.2 kg/m²). Blood pressure on home monitoring averages 120/78 mmHg (on telmisartan 40mg). Dilated fundus examination negative for retinopathy.

LABORATORY MEASUREMENTS (VISIT-OVER-VISIT COMPARISON)

Biomarker / Investigation	Oct 2026 (Baseline)	Jan 2027 (Current)	Trajectory / Delta	Reference Interval
Fasting Plasma Glucose	**142.0 mg/dL**	**96.0 mg/dL**	**Improved (-46.0 mg/dL)**	70.0 - 99.0 mg/dL
Glycated Hemoglobin (HbA1c)	**11.2 %**	**6.6 %**	**Marked Improvement (-4.6 %)**	< 5.7 % (non-diabetic)
Blood Pressure (Systolic / Diastolic)	**148/92 mmHg**	**120/78 mmHg**	**Normalized (-28/-14 mmHg)**	< 120 / < 80 mmHg
Total Cholesterol	**238.0 mg/dL**	**184.0 mg/dL**	**Optimal (-54.0 mg/dL)**	< 200.0 mg/dL
LDL Cholesterol (Atherogenic)	**152.0 mg/dL**	**96.0 mg/dL**	**Target Achieved (-56.0 mg/dL)**	< 100.0 mg/dL
Serum Creatinine (Preserved)	1.0 mg/dL	**0.9 mg/dL**	Stable Renal Function	0.7 - 1.3 mg/dL
Calculated BMI	**27.8 kg/m²**	**26.2 kg/m²**	**Reduced (-1.6 kg/m²)**	18.5 - 24.9 kg/m²
Urine Albumin:Creatinine Ratio	**84.0 mg/g**	**22.0 mg/g**	**Resolved (<30 mg/g)**	< 30.0 mg/g

TREATMENT PLAN & CLINICAL RECOMMENDATIONS

1. Outstanding therapeutic response: Patient has successfully transitioned from severe ketotic decompensation to near-target glycaemic control.
2. Step-down insulin therapy: Reduce bedtime Glargine to 12 units. Discontinue pre-meal Aspart; continue Metformin SR 500mg BID.
3. Cardiovascular risk protection: Continue Telmisartan 40mg OD and Atorvastatin 20mg OD (LDL now <100 mg/dL target met).
4. Follow-up: Repeat HbA1c and renal function panel in 3 months (April 2027).

Dr. Neha Example, MD, DM (Endocrinology)

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